Prior Authorization Request Packet

Case pa_db2e223b · Aetna · MRI lumbar spine without contrast (72148) · Policy Aetna CPB 0236
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1. Letter of medical necessity

October 11, 2026

Aetna Utilization Management / Prior Authorization Department

RE: Daniel Okafor, DOB 1979-03-14, Member ID AET-448120933
Requested service: MRI lumbar spine without contrast (CPT 72148)

To the Medical Director:

I am writing on behalf of Dr. Priya Raman to request prior authorization for the lumbar spine MRI above. The request is reviewed against Aetna CPB 0236 (https://www.aetna.com/cpb/medical/data/200_299/0236.html).

Clinical summary: Mr. Okafor is a "47-year-old male" with low back pain and right leg symptoms. The note states: "Pain began approximately 10 weeks ago after lifting boxes at work." Pain is "7/10, worse with sitting and forward flexion." Examination found "Positive straight leg raise on the right at 40 degrees." It also found "Mild weakness of right plantar flexion, 4/5." and "Decreased sensation to light touch along the lateral right foot." The note also records "Gait antalgic." and "No clonus."[quote removed by verifier]"Right S1 lumbar radiculopathy, likely L5-S1 disc herniation, refractory to 8 weeks of conservative management with objective neurologic deficit."[quote removed by verifier]"He describes sharp pain in the right buttock radiating down the posterior thigh and calf into the lateral foot, consistent with an S1 distribution."[quote removed by verifier]"Diminished right ankle reflex (1+ compared with 2+ on the left)."[quote removed by verifier]"Mild weakness of right plantar flexion, 4/5."[quote removed by verifier]"Completed 8 weeks of conservative therapy beginning 07/28/2026"[quote removed by verifier]"naproxen 500 mg twice daily", "cyclobenzaprine 10 mg at bedtime", and "activity modification". It also included "a formal physical therapy program at Bayview PT (12 sessions, 08/04/2026 through 09/22/2026)". The documented 8.0 weeks exceeds the 6 weeks required. The note states: "He reports no meaningful improvement despite full adherence."

The policy also lists indications that do not require a conservative therapy trial. These are suspected cord or cauda equina compression, rapidly progressing or major motor deficit, suspected infection, suspected primary tumor or metastasis, and suspected fracture. None of these is documented as the basis for this request, and I am not relying on them. For context, the note records "He denies saddle anesthesia" and no "bowel or bladder dysfunction", "fever", or "unexplained weight loss". It also states "No history of cancer." The weakness recorded is mild at 4/5, and the note does not describe it as rapidly progressing.

Conclusion: Mr. Okafor meets all three criteria for lumbar MRI in radiculopathy under CPB 0236.

He has dermatomal radicular pain, objective reflex, motor, and sensory findings, and no improvement after more than 6 weeks of documented conservative therapy. I respectfully request approval of CPT 72148.

Sincerely,

Priya Raman, MD
NPI 1891234567
Ordering Physician

2. Criteria checklist with verified evidence

MET aetna-72148-c1: Low back pain with radiculopathy: radicular leg pain in a nerve root (dermatomal) distribution.

"He describes sharp pain in the right buttock radiating down the posterior thigh and calf into the lateral foot, consistent with an S1 distribution."

MET aetna-72148-c2: Pain plus objective findings of motor or reflex changes in the specific nerve root distribution.

"Diminished right ankle reflex (1+ compared with 2+ on the left)."

MET aetna-72148-c3: No improvement after 6 weeks of conservative therapy (conservative therapy = moderate activity, analgesics, non-steroidal anti-inflammatory drugs, muscle relaxants).

"Completed 8 weeks of conservative therapy beginning 07/28/2026"

code check: 8.0 weeks documented vs 6 required

MISSING aetna-72148-r1: Clinical suspicion of a spinal cord or cauda equina compression syndrome (e.g., saddle anesthesia, new bowel or bladder dysfunction, urinary retention, bilateral leg weakness).

MISSING aetna-72148-r2: Rapidly progressing neurological deficit, or major motor weakness.

MISSING aetna-72148-r3: Suspected infectious process (e.g., osteomyelitis, epidural abscess of the spine or soft tissue), such as back pain with fever or intravenous drug use.

MISSING aetna-72148-r4: Primary spinal bone tumors or suspected vertebral, paraspinal, or intraspinal metastases (e.g., new back pain with a history of cancer or unexplained weight loss).

MISSING aetna-72148-r5: Suspected spinal fracture and/or dislocation secondary to trauma (if plain films are not conclusive).

3. Source visit note

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CHIEF COMPLAINT: Low back pain radiating down the right leg.

HISTORY OF PRESENT ILLNESS: 47-year-old male presents for follow-up of low back pain with
right-sided radicular symptoms. Pain began approximately 10 weeks ago after lifting boxes at
work. He describes sharp pain in the right buttock radiating down the posterior thigh and
calf into the lateral foot, consistent with an S1 distribution. Pain is 7/10, worse with
sitting and forward flexion. He denies saddle anesthesia, bowel or bladder dysfunction,
fever, or unexplained weight loss. No history of cancer.

TREATMENT TO DATE: Completed 8 weeks of conservative therapy beginning 07/28/2026: naproxen
500 mg twice daily, cyclobenzaprine 10 mg at bedtime, activity modification, and a formal
physical therapy program at Bayview PT (12 sessions, 08/04/2026 through 09/22/2026) focused
on core stabilization and McKenzie extension exercises. He reports no meaningful improvement
despite full adherence.

PHYSICAL EXAM: Positive straight leg raise on the right at 40 degrees. Diminished right ankle
reflex (1+ compared with 2+ on the left). Mild weakness of right plantar flexion, 4/5.
Decreased sensation to light touch along the lateral right foot. Gait antalgic. No clonus.

ASSESSMENT: Right S1 lumbar radiculopathy, likely L5-S1 disc herniation, refractory to 8
weeks of conservative management with objective neurologic deficit.

PLAN: MRI lumbar spine without contrast (CPT 72148) to evaluate for disc herniation and nerve
root compression and to guide possible epidural steroid injection or surgical referral.
Continue home exercise program.
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